Colombia

INSTITUTIONAL RESPONSE 1 OF 2 · INDEPENDENT RECORD

06 / 16
REGIONAL SURVEY 2026

Public Health EOC - National Institute of Health (INS)

2017	24/7 continuous	< 2 hours	National	10
Established	Operating mode	Activation time	Mandate	24/7 duty staff

CAPABILITY SNAPSHOT

	Response	Responding-country mean (n=15; countries equally weighted)
Governance & legal	65	
Operations & decisions	83	
Human resources	85	
Information management	85	
Coordination	33	

HAZARDS COORDINATED

Geological / weather	>5
Biological / epidemic	>5
Social / human-caused	1-2
Technological	None

Frequency bands: none · 1–2 · 3–5 · >5

INSTITUTIONAL & OPERATIONAL ATTRIBUTES

Legal framework	Developing	Surge staffing	Functional
National plan	No	Trained roster	Updated
Formal SOP	Yes	Team deployment	Leads
Operating finance	Functional	Intersectoral links	Health sector only
Incident management	Used	DRM integration	Limited
Incident action plan	Systematic	Intl. cooperation	Via mechanism
Risk assessment	Systematic	Information tools	Interoperable
Strategic committee	Unknown	SITREP process	Formal & regular
EOC coordinator	Informal	Situation room	Integrated
Documented roles	Formal	Logistics readiness	Consolidated

OPERATIONAL RESOURCES

● Dedicated room	○ Shared room
● Virtual platform	● Communications
● Situation displays	● Backup power
● Backup internet	● Workstations

● Reported ○ Not selected – Missing

TRAINING

✓ Completed ★ Priority to strengthen

EOC activation ✓	Incident mgmt ✓
Information mgmt ★	Coordination ★
Damage / needs ★	Situation analysis ★
Response teams ★	After-action review ✓ ★

REPORTED STRENGTHS

- Trained multidisciplinary personnel
- Established response and monitoring capabilities
- Institutional commitment to emergency coordination

IMPROVEMENT PRIORITIES

- Improve interoperable information systems
- Strengthen rapid-response team readiness
- Formalize procedures and communications capacity